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Understanding Nerve Pain in Your Arms and Legs (Radicular Pain): Patient Information Leaflet

This leaflet is designed to help you understand the cause of pain that travels down your arms or legs, the various treatment options available, and what you can expect for your recovery. Please use this information to support discussions with your healthcare team.

What is Radicular Nerve Pain?

Radicular pain is a specific type of pain that “radiates” or travels along the path of a nerve. It feels like a sharp, shooting, burning, or electric shock-like sensation.

The Cause

This pain occurs when a nerve root in your spine is irritated or compressed. Think of nerve roots as the main electrical cables that branch out from your spine to your entire body.

Location Matters

In the Neck (Cervical Spine):

Irritation causes pain to radiate into your shoulder, arm, or hand. This is often called a “pinched nerve” in the neck.

In the Lower Back (Lumbar Spine):

Irritation causes pain to radiate into your hip, buttock, leg, or foot. This is commonly known as “sciatica.”

Common Sources of Irritation:

- A herniated or “slipped” disc (the most common cause).
- Spinal stenosis (narrowing of the spinal canal).
- Bone spurs from arthritis or degenerative changes.

It is important to know: You can have radicular pain without any significant nerve damage.

Radicular Pain vs. Radiculopathy: What's the Difference?

While these terms are related, they describe different problems

Radicular Pain

Refers to the sensation of pain travelling down the nerve pathway. The nerve is irritated and sending pain signals.

Radiculopathy

Refers to a functional problem with the nerve. This means the compression is strong enough to cause a loss of function, leading to;

- Numbness or loss of feeling.
- Tingling (pins and needles).
- Muscle weakness in the arm, hand, leg, or foot.

You can have one or both conditions at the same time. The presence of weakness or numbness (radiculopathy) is a key detail your doctor will assess.

What is the Overall Prognosis (Outlook)?

The outlook for most people with radicular nerve pain is very good.

Natural Recovery

A significant number of people see their symptoms improve significantly or resolve completely with time and conservative management alone. This is often because a herniated disc can naturally shrink (resorb) over several weeks to months.

Influence of Factors

Your individual prognosis depends on the underlying cause, your overall health, and how consistently you follow your treatment plan, particularly physical therapy.

Conservative (Non-Surgical) Treatment Options

Most of the radicular pain improves with conservative care. The goal is to reduce inflammation and pain, allowing the body to heal naturally.

Medication

- Anti-inflammatories: (e.g., Ibuprofen, Naproxen) to reduce swelling around the nerve.
- Neuropathic Pain Medications: (e.g., Gabapentin, Pregabalin) specifically target nerve pain.
- Muscle Relaxants: Can help with associated muscle spasms.

Physical Therapy

A cornerstone of treatment. A therapist will teach you

- Specific exercises and stretches to relieve nerve pressure.
- Core and neck strengthening to support your spine.
- Proper posture and body mechanics to prevent re-injury.

Activity Modification

Avoiding activities that trigger pain (like certain arm movements or lifting) but staying active. Prolonged rest is generally not recommended.

Epidural Steroid Injections

If pain is severe or not improving, an injection can be a very effective option to deliver anti-inflammatory medicine directly to the source of the problem. These are minimally invasive procedures where powerful anti-inflammatory medication (corticosteroid) is delivered directly to the area around the irritated nerve.

Foraminal Epidural Injection (Transforaminal Injection)

This is often considered the most direct and targeted injection for a single irritated nerve root, whether in the neck or lower back.

The “Target”

The intervertebral foramen – a small, bony “keyhole” on the side of the spine where the individual nerve root exits.

The Procedure (in simple terms)

- You will lie on your stomach (for low back) or on your back (for neck), and the skin will be numbed with local anaesthetic.
- Using live X-ray guidance (fluoroscopy), your doctor will safely guide a very thin needle directly to the opening (foramen) where the specific irritated nerve is located.
- A small amount of contrast dye is injected to confirm the medication will flow around the targeted nerve root.
- A mixture of a long-lasting corticosteroid and a local anaesthetic is injected.

The Goal

To bathe the inflamed nerve root in a high concentration of anti-inflammatory medicine, dramatically reducing swelling and pain.

Prognosis & Success Rates of Foraminal Epidurals:

- Short-Term Success: Studies show that a significant majority of patients (rates are generally between 70% and 85%) experience meaningful pain relief within the first 1-4 weeks after the injection.
- Long-Term Success: The primary goal is to provide a “window of relief” (often lasting 3 months or more) during which you can effectively participate in physical therapy. The long-term success

is highly dependent on using this pain-free period to strengthen your body and improve spinal mechanics. Many patients experience lasting relief after one or a series of injections.

- It's important to understand: The injection is not a "cure" for the underlying structural issue. It is a powerful tool to control inflammation and break the cycle of pain, making rehabilitation possible. Success is defined by a significant reduction in pain and an improved ability to function.
- In some cases, the foraminal epidurals do not help in reducing the pain, in that case a referral to a surgeon will be made.
- Occasionally, these injections can cause a flare-up, which is treated with medications and rest.
- Rare side effects include further damage to the nerve or the spinal cord.

Other Injection Approaches (like Interlaminar epidurals) may also be used depending on your specific anatomy and the location of the problem. Your doctor will recommend the best approach for you.

When is Surgery Considered?

Surgery is typically reserved for when conservative treatments (including injections) have failed to provide adequate relief, or if you develop specific warning signs.

Reasons for surgery may include:

- Severe or worsening pain that does not improve after 6-12 weeks of non-surgical care.
- Progressive weakness in the arm, hand, leg, or foot.
- Numbness that is getting worse.

Surgical Options (An Overview)

Your surgeon will recommend a procedure based on the specific cause and location of your nerve compression.

For the Neck (Cervical Spine)

- Anterior Cervical Discectomy and Fusion (ACDF): This is the most common surgery for a pinched nerve in the neck. The surgeon approaches from the front of the neck, removes the problematic disc that is pressing on the nerve, and then fuses the two vertebrae together with a bone graft and a small metal plate to stabilize the area.
- Laminectomy/Foraminotomy: Performed from the back of the neck to remove bone spurs or a small piece of bone to open up the space for the nerves.

For the Lower Back (Lumbar Spine)

- Discectomy: The most common surgery for a herniated disc. The surgeon removes the portion of the disc that is pressing on the nerve.
- Laminectomy: Used for spinal stenosis. The surgeon removes a section of the bony arch (lamina) at the back of the vertebra to create more space for the nerves.
- Spinal Fusion: This surgery joins (fuses) two or more vertebrae together permanently with metal rods and screws. It is used to stabilise the spine after removing a significant amount of bone or disc, or if there is abnormal movement (instability).

Questions? Contact Your Care Team

Before/after procedure:

 782-377-7813

 Info@redsandspm.com

In an emergency

→ Call 911, or visit your local emergency department

Disclaimer

This leaflet is for informational purposes only and does not replace professional medical advice. Diagnosis and treatment should be made by a qualified healthcare provider.